23. PRACTICAL NOTES BEFORE APPROACHING THE LIVER

DURATION : 06:13 STUDY SHEET

COURSE SUMMARY

In this session, practitioners will learn to explore the liver, taking into account its integration into the whole body and its relationships with the heart and surrounding structures. Concepts of embryonic motility are at the heart of the practice, where the technique will begin with neutral contact and progress towards a deeper approach, thus promoting primary respiration and the emergence of a therapeutic process. Participants will be guided on the importance of working in connection with embryonic fulcrums, particularly the midline and the heart, in order to energise the vascular and neurological systems, and thereby improve the effectiveness of their treatment.

PRACTICE BEFORE APPROACHING THE LIVER

In this session, we will explore the **liver** in relation to its **global environment**. It is essential not to adopt an overly **selective** or **local** approach.

The liver plays a central role in the organisation of the **omental bursa**, the positioning of the **stomach**, the axis of the **spleen**, and the **greater omentum**. We will practice on the liver while considering its relationship with the **heart** and the omental bursa, using an appropriate handhold.

The objective is to allow the liver to regain its **motility**, as it is this motility that creates its function. We will begin with a **neutral** and superficial contact, which will progressively deepen as the **primary respiration** manifests. The meeting of these two neutrals is what brings forth the **therapeutic process**.

The liver is crucial in the organisation of the embryo, particularly in the **transverse** axis and the establishment of the **gastro-duodeno-pancreatico-splenic** axis, thus managing all movement. This tilting, which we previously discussed with the **venous** and **arterial** networks, is an essential developmental movement for growth. The liver moves upon itself, not by rotation, but by **displacement**, which gives it a developmental force.

This motility of embryonic origin is fundamental. When we place our hands, we must position ourselves in a **fulcrum** where the entire system can develop. If we limit ourselves to a local approach, we lose sight of the original fulcrum.

The closer we get to the embryonic fulcrum, the more we access this force, and the more the body can provide us with information, because it knows it. A high frequency promotes **regenerative power**. Consider the word "fulcrum" as a **function** rather than a mere point of support.

The **midline**, for example, is a fulcrum for the totality. We have identified the heart area as an embryonic fulcrum, which cannot be reduced to a simple point.

Working on motility related to **brain** development can serve as a tool. Let's recall the curvatures, this "tai chi" of the brain:

- **Expansion**
- **Cephalic flexion**
- **Cervical flexion**
- **Pontine flexion**
- **Telencephalisation**

The telencephalisation phase is crucial, as it induces the straightening and establishment of the **anterior white line**. This involves the formation of the **star**.

We can work solely on motility or use **specific points** such as:

- The tip of the **monotocorte**
- The **supraocciput**
- The base of the **occiput**
- The **presphenoid**

These points are related to these developments. For example, the tip of the monotocorte is an important reference point in **submesencephalic flexion**.

I can thus search for the **still point** in the developing system, depending on the points of support on which I position myself. Certain phases will occur simultaneously, illustrating the **synchronicity** of development.

Regarding brain development, an important vestige is found at the level of the **mesenteric system**. The **vitelline** or **mesenteric** axis is crucial for re-energising a developing brain.

From a vascular perspective, it is important to note that the brain is entirely dependent on the heart for its function and formation. The heart continuously informs the brain, and a **pulsation** is present within it.

Keep in mind the image of the **septum transversum** coiling with the establishment of the liver, integrated with the **vitelline vesicle**. The establishment of the **amniotic cavity** is the result of our previous work.

There is a movement of developmental motility generated by the liver, which organises the entire **hepato-gastro-duodeno-pancreatico-splenic** system. We observe the tilting of the heart on its left axis, an inverse movement at the global hepatic level, and a counter-rotation on the digestive plane.

We will first work on this large **subdiaphragmatic** organisational centre, by establishing the omental bursa. Then, we will examine the relationship between the liver and the heart, which will be our two working points.

We have the **cardiac imprint**, the **pulmonary imprint**, and the enormous **hepatic imprint**. We will perform work on the liver in relation to the establishment of the heart and the intestine, adopting a **biodynamic** approach.

This means we will place our hands and allow the movement to occur. We may have a movement that seeks or a movement that expresses its originality. We will provide this point so that the system can reorganise itself.

The liver is a **specialisation** to be considered as a **fractal expansion** of the intestine. It consists of a set of **bile ducts** with two types of information:

- **Vascular** type information, corresponding to the **portal** type liver.
- A very red face may indicate a **mesodermal** type hepatic problem.
- A very yellow face may indicate an **endodermal** type hepatic problem.

These two approaches allow for a complementary understanding of the system.